



OFFICIAL

NMHS MHS PROCEDURE

Duress Alarm Systems

Scope (Audience)	All NMHS Mental Health Services (MHS) Workers
Scope (Area)	All NMHS MHS Areas
Summary	In accordance with the Work Health and Safety Act 2020 and the NMHS Duress Alarm Policy this procedure sets out the mandatory requirements relating to the management, roles and responsibilities and maintenance of duress alarms across North Metropolitan Health Service (NMHS) Mental Health Services (MHS).
Risk Rating	Medium
NSQHS Standards	Clinical Governance (Std 1) Communicating for Safety (Std 6)
Policy Sponsor	Emergency Management
Policy Contact	Manager Emergency Management
Version	1.0
Review Date	[Policy Unit to complete]

COMPLIANCE WITH THIS DOCUMENT IS MANDATORY

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1. Aim

In accordance with the [Work Health and Safety Act 2020](#) and the [NMHS Duress Alarm Policy](#) this procedure sets out the mandatory requirements relating to the management, roles and responsibilities and maintenance of duress alarms across North Metropolitan Health Service (NMHS) Mental Health Services (MHS).

2. Risk

Non-compliance with this policy will impact on the following risk categories:

Culture	☐	Organisational Development	☐
Emergency Management	☐	Policy & Legislative Compliance	☐
Fiscal Responsibility	☐	Quality of Patient Care	☐
Governance & Accountability	☐	Research & Innovation	☐
Information Communications Technology	☐	Staff Safety & Wellness	☒
Facility Management	☒	Workforce Planning	☐
Medical Equipment	☐	Other	☐

3. Principles

- All workers have a general duty of care to maintain their own safety and health at work, and look out for the safety of colleagues, patients, and visitors. This includes complying with all Workplace Health and Safety instructions and making use of safety systems and procedures provided by MHS.
- The duress response system is provided to people in areas risk assessed as requiring fixed point and/or personal duress alarms (PDA) to increase safety and security measures.
- Workers must never hesitate to activate a fixed or personal duress alarm if their personal safety, or the safety of others, is threatened.
- Workers are to comply with site requirements for calling a Code Black, as outlined in the [Emergency Procedure Manual Code Black Action cards](#).
- All NMHS MHS sites **must** have a duress system (fixed and/or personal pendants) which include a localised process for response and escalation.

4. Procedures

4.1 Hospital Duress Alarms

NMHS MHS workers must call for assistance and activate the duress or personal alarm, where available, when workers experience or witness:

- Violent behaviour.
- Aggression or threats towards workers / patients / visitors.

Workers should ensure their own safety. Then when it is safe to do so, workers should summon assistance by:

- Calling '55' or declaring a Code Black (where appropriate).
- Activating a Duress Alarm.
- Calling site Security directly (where available).

Duress alarms may either be fixed hard wired or personal mobile alarms.

Response will be provided by workers within the duress response group, where implemented. Only PART trained workers with appropriate skill in dealing with violence and aggression will be allocated to the duress response group.

4.2 Community Duress Alarms

For community clinics that do not have a '55' response there shall be a clear internal response process and guidance as to when Police assistance will be sought.

Workers who undertake community visits are to be trained in the use of the personal duress alarm device provided to them by the managers of the area.

Workers must **ALWAYS** carry appropriate communication equipment including a personal duress alarm in accordance with MHS Home and Community Visiting Procedure.

5. Roles and Responsibilities

5.1 Managers and Supervisors

Dependent upon the site / service the manager / supervisors responsibilities may include the following:

- Ensuring that day to day implementation and maintenance of the duress system in their departments, including testing, is complied with.
- Ensuring that a risk assessment is undertaken for the work area to assess risk and determine duress alarm requirements for each workgroup, including contractors.
- Ensuring that accurate records of duress alarms are maintained.
- Ensuring appropriate workers are educated on the use of the duress system.
- Ensuring appropriate workers are issued with and educated on the use of personal duress alarms and the mandatory requirement that workers have the alarm on them at all times.
- Ensuring workers **ALWAYS** carry appropriate communication equipment including a personal duress alarm.
- Coordination of the workers responding to the duress alarm.
- Reporting of duress incidents.
- Ensuring that workers incidents are reported and investigated in accordance with the [NMHS Hazard/Incident Reporting and Investigation Policy](#).
- Ensuring that patient incidents are reported and investigation in accordance with the MP 0122/19 [WA Health Clinical Incident Management Policy](#) and the [NMHS Clinical Incident Management Policy](#).
- Reporting problems with the duress system to the appropriate authority.
- Ensuring workers carry out testing of relevant duress systems.
- Managers / supervisors are to ensure that external contractors are informed of and assigned a duress alarm, where necessary.
- Ensuring where required that workers always have the alarm on them.
- Managers are to develop and implement an induction package for new starters and contractors to ensure that safe systems of work are provided to all workers in the department, including records of training and instruction.

5.2 NMHS Facilities Management (FM)

The NMHS FM responsibilities include the following:

- Procurement of Duress Alarms to be issued to workers within NMHS MHS.
- Provisioning of Duress Alarms for use by workers within NMHS MHS.
- Procurement and commissioning of the Personal Duress Alarms.
- Managing and maintaining Duress Alarms whilst issued to workers within NMHS MHS including:
 - developing processes and providing requisite Equipment to reprogram Duress Alarms in the field as required to deliver the Services; and
 - providing the service with the maintenance reports once contractor has been onsite.

5.3 Worker/s

Worker/s responsibilities include the following:

- Familiarising themselves with the duress system in their work environment including the location of fixed-point duress alarms.
- Appropriate testing of personal duress alarms.
- Immediately reporting any failure or malfunction of the duress system to line management.
- **Carrying allocated personal duress devices with them at all times.**
- Respond to the activation of a duress alarm by another worker, as per local duress response procedures.
- Ensuring that worker incidents are reported in accordance with the [NMHS Hazard/Incident Reporting and Investigation Policy](#).
- Taking appropriate care and returning the duress alarm when it is no longer needed.

5.4 Issuing and Recording

Areas with personal duress alarms in use shall develop protocols in place to manage the issuing and recording of duress alarms and these will be documented in a register. At Graylands Hospital (Acute and HECS) a RedCap system is in place and is maintained by the Adult Leadership team.

Managers shall ensure duress alarms will be issued to all workers working in environments where they may be required.

The manager / supervisor of the department issuing the alarms will ensure that an accurate record is maintained.

Managers are responsible for the maintenance and replacement costs of any damaged, stolen or lost duress devices and will need to provide a cost centre prior to any replacement duress alarms being received.

Workers must not swap or share duress alarms once issued unless the department has a comprehensive way of tracking who the duress alarm has been issued to and can advise the monitoring company in a fluid manner.

5.5 Education and Training

Managers are responsible to ensure all relevant persons who work/access in a ward / unit / Community setting where duress alarms are located / provided shall receive training in their use.

All NMHS MHS workers are required to attend annual [Mandatory Training](#) for the management of violence and aggression.

5.6 External Contractors

External contractors operating in areas covered by a duress alarm system should be provided the induction to the department training, including orientation to the duress system, and assigned a duress alarm. Contractors shall not be expected to respond to duress systems (excepting Security Contractors) Isolation of Duress Systems

If an area or system of duress is required to be deactivated and isolated, the following must be completed:

- Risk assessment of duress isolation, including consideration of risk of extended isolation/deactivation
- Activation of suitable alternative (e.g. personal instead of fixed)
- Reduction or closure of service until duress system returned to service
- Communication to all affected workers, including Security and Emergency Management, of the isolated area, predicted length of isolation, alternate control measures and any other relevant matters

Approval to isolate a duress system, excepting for regular, planned maintenance, can only be approved by Tier 3 or above.

Isolation may occur due to system faults, service interruption or malicious use of duress fixtures. Isolations shall be as short as reasonably practicable, until the hazard is resolved or risk reduced.

5.7 Duress Alarm Testing Hospital Based Sites

The frequency of duress alarm testing will be determined upon the installation of the system.

This is to be communicated to all relevant site-based managers and supervisors. As a minimum duress alarm will be tested within the following timeframes:

- Fixed hard wired alarm, six (6) monthly performed by Facilities Management.
- Personal Duress alarms, three (3) monthly check and at every commencement of the workers shift.

Adequate record keeping will be maintained on all testing of fixed duress alarms systems by Facilities Management. Adequate record keeping of personal and internal alarms should be maintained by managers of the departments. All site / service duress alarm system testing should be audited annually, with the results submitted to the respective site Emergency Management Committee upon request.

Quarterly testing of the Home and Community Visiting PDA devices (Sofi Hub) is required, ensuring that accurate records of duress alarms are maintained.

5.8 Community Duress Alarms

The frequency of duress alarm testing will be determined by the relevant managers of community-based workplaces. The recommendation is monthly testing of the personal duress alarms.

Adequate record keeping should be maintained on testing of duress alarms systems by managers of the departments where personal duress alarms are provided.

Quarterly testing of the Home and Community Visiting PDA devices (Sofi Hub) is required, ensuring that accurate records of duress alarms are maintained.

6. Fault Reporting

6.1 Hospital Duress Alarms

Faults should be reported immediately to site / service facilities / engineering departments by responsible manager, or if noticed by Security, Security should raise for remedial action.

The Work Health and Safety Department and Emergency Management should be contacted to discuss strategies to maintain a safe working environment while the duress system is out of order.

6.2 Community Duress Alarms

Faults should be reported immediately to site / service facilities / engineering departments by responsible manager of Community setting including raising an Enterprise Management Procurement and Costing (EMPAC) request.

Faulty Community Duress Alarms shall be reported direct to supplier for rectification/remediation or replacement by Facilities Management.

Work Health and Safety and Emergency Management should be contacted to discuss strategies to maintain a safe working environment while the faulty duress solution is out of order.

7. Compliance, Monitoring and Evaluation

Legislative obligations relating to the management of violence and the organisation's responsibilities for providing a safe working environment under the [Work Health and Safety Act 2020](#) and the [MP 0180/23 Work Health and Safety Management Policy](#).

Each NMHS MH Service / Site must be able to demonstrate compliance with this policy by providing evidence that the audits/ monitoring requirements outlined in this procedure were met upon request.

Site / service executive / NMHS FM / management teams will be responsible for ensuring that all compliance audit and reporting is undertaken.

Compilation of site / service compliance reports may be submitted to the NMHS MHS Emergency Management Committee and relevant NMHS MH Service / Site-based Governance Committee/s.

This procedure will be evaluated as required to determine its currency, relevance, effectiveness, efficiency, and sustainability. At a minimum, it will be reviewed 3 years using a risk management approach in accordance with the [NMHS Policy Governance Framework](#).

8. Definitions

Terms	Definitions
Code Black	A Code Black is a call for assistance, when any individual feels themselves or others are at risk from an act of aggression, or there is a perceived risk to property. There are five different types of Code Black that can be activated 'Personal Threat', 'Self Harm', 'Armed-Hold-up', 'Alpha' or 'Bravo'.
Duress Alarm	A device that, when activated, indicates a person is under threat and requires immediate assistance. • Fixed Point Duress Alarm: Fixed point duress alarm buttons are strategically located throughout a facility often fixed under desks or to walls, which are hard-wired to a pager alert system. • Personal Duress Alarm (PDA): A wireless device, worn or carried by a person, for mobile activation of a duress alarm inside the workplace and/or while on home / community visits.
Emergency Response Team (ERT)	Allocated workers whose role is the response to emergency codes within the hospital.
Pager Alert System	An enterprise communication system, monitored by the relevant Emergency Response Team (ERT) workers, which provides real time alerts to facilitate immediate and appropriate response to duress alarms.
Triple Zero "000"	Emergency Telephone Number for workers in the community, or sites where there is no Code Black response available or "55".
Worker	According to the Work Health and Safety Act 2020, a person is a worker if the person carries out work in any capacity for NMHS, including work as – a) an employee; or b) a contractor or subcontractor; or c) an employee of a contractor or subcontractor; or d) an employee of a labour hire company who has been assigned to work in NMHS; or e) an outworker; or f) an apprentice or trainee; or g) a student gaining work experience; or h) a volunteer; or i) a person of a prescribed class.
Workplace	An area or place or vehicle, where a worker goes or is likely to be while work is carried out on behalf of the NMHS and includes but is not limited to hospital premises, laboratories, workshops, training rooms, on-site and off-site facilities and offices, and patient's homes.

Refer to [NMHS Policy Document Glossary of Terms](#) for any other frequently used terms.

9. Resources

- [NMHS Health, Safety and Wellbeing Health Point](#)
- [NMHS Hazard Incident Form](#)
- [NMHS Workplace Violence and Aggression Incident Form](#)

10. Related Documents

Legislation	• Work Health and Safety Act 2020 • Work Health and Safety (General) Regulations 2022
WA Health mandatory policies	• MP 0122/19 WA Health Clinical Incident Management Policy • MP 0180/23 Work Health and Safety Management Policy
NMHS Policy documents	• NMHS Home and Community Visiting Policy (coming soon) • NMHS Duress Alarm Policy • NMHS Working Alone Policy • NMHS Incident Hazard Reporting and Investigation Policy • NMHS SPE17: Security Policy • NMHS Prevention & Management Workplace Aggression • NMHS Occupational Safety and Health Working Alone Policy • NMHS Employee Induction and On-boarding Policy •
NMHS MHS Policy documents	• Home and Community Visiting Procedure (coming soon)
Standards	• Australian Standards AS 4485.1-1997 Security for Health Care Facilities • AS 4485.2-1997 Security for Health Care Facilities Part 2: Procedures Guide • Australasian Health Facility Guidelines (HFG) Part C, Design for Access, Mobility, OHS and Security, Section 790 - Security Precautions, revised 2016. • NSQHS Standards - Clinical Governance Standard. • National Safety and Quality Mental Health Standards for Community Managed Organisations

11. Document Control

Document Version Control			
Date	Version	Author	Notes
28/08/2024	V1.0	Emergency Manager MHPHDS	Initial endorsement
31/10/2025	V2.0	Emergency Manager MHSDHS	Reviewed and updated policy to procedure to align with the NMHS Duress Alarm Policy.

This document can be made available in alternative formats on request for a person with a disability.

Within the Department of Health WA, the term Aboriginal is used in preference to Aboriginal and Torres Strait Islander, in recognition that Aboriginal people are the original inhabitants of Western Australia. No disrespect is intended to our Torres Strait Islander colleagues and community.

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